Worked Example 2 — The Buyer Stack Worksheet, filled in for a pilot that stalled

A companion to Instrument 1 of The Operator's Thesis (Chapters 4–5), with Instruments 5, 6 and 8. Hypothetical composite: the organization, the people, the vendor and the figures below are invented for teaching and are labeled illustrative. Nothing here describes a real deployment, a real vendor, or any employer, client, deal or fund. Version 1, October 2026.

The situation (illustrative)

Northgate Regional is a composite: a 450-bed regional system with twenty-two primary-care clinics and a patient-message problem — the inbox grew 40 percent in three years and the nurses who triage it are leaving. A vendor offered a free six-month pilot of an AI tool that reads incoming patient messages, drafts a triage category and a reply, and routes the message to the right queue. Six clinics started in January. In month four the pilot stalled: the drafts were still being produced, the nurses had quietly stopped using them, the medical director who had championed the tool had taken a different role, and nobody could say who would decide whether to continue.

The vendor called it a change-management problem. The nurses called it more work. Both were partly right, and the Buyer Stack worksheet shows why, because the empty cells are the finding.

The worksheet, filled in (as it stood in month four)

Seat	Who, by name	What they need before yes	What they risk by yes
Champion	*Unfilled since month three* — Dr. Lena Okafor, primary-care medical director, championed the pilot until she left the role; no successor was named	Evidence the tool reduced inbox time for her clinicians without harming patients	Her credibility with the clinics; the political capital she spent getting six sites to say yes
Sponsor	*(never assigned)* — the chief medical officer "supported" the pilot but never accepted the role	A sponsor would need the burden map, the safety architecture (Instrument 6) and a renewal plan before lending the executive's name	Being accountable for a tool that touches clinical triage
Signer	Marcus Bell, vice president of supply chain, who signed a $0 pilot agreement	He treated the $0 price as a reason not to assemble the evidence. He still needed it: a review of the data-use terms, the security assessment, the workflow burden, the support obligations, liability, and the paid terms that would follow the pilot — none of which a zero on the invoice supplies	A term sheet for year two that nobody has seen, with a data-use clause nobody has read
Budget owner	*(never assigned)* — no one owned the pilot's internal resource budget or the continuation decision, and year two was never priced	The Price-Plus-Burden sheet (Instrument 3): the pilot consumed an analyst's quarter, a nurse educator's month and six clinic managers' attention; the year-two invoice is unknown	Owning a line that appears in month seven with no evidence behind it

Seat	Who, by name	What they need before yes	What they risk by yes
Implementer	Priya Natarajan, ambulatory informatics manager — *learned of the pilot at the kickoff meeting*	A charter (Instrument 8), an integration plan, an exception path, a training plan; none existed in January	Inheriting a deployment designed without her, and the exception queue that came with it
User(s)	The triage nurses and medical assistants in six clinics; the physicians who receive the routed messages	A draft they can trust more often than they must correct; a review surface that makes the correction faster than writing from scratch; a way to flag the drafts that are wrong in a way that matters	Verifying every draft (the new work the demo never showed); routing errors that reach a patient; being measured on a tool they did not choose
Renewal owner	*(never assigned)* — no one was named, no date was set, no evidence was specified	A renewal decision needs a date, an owner and the evidence the institution will require (Instrument 11)	The default outcome: a renewal that happens by inertia, or a cancellation that happens by neglect, with no record of which was right

Scoring is unnecessary. Three rows were never formally assigned — sponsor, budget owner and renewal owner — and the champion's row became unfilled when the medical director left in month three. At month four, four of the seven functions have no current owner; the signer is named, but signing a zero-price agreement did not establish that the evidence had been reviewed. That is the risk report. What the worksheet shows is missing ownership and an unpriced burden on the nursing workflow. What it does not show is whether the model was any good: that question is still open at month four, and the restart below has to answer it rather than assume it.

The three diagnostic questions

Which seat is doing double duty, and can they? The medical director informally carried the coordination and follow-up a sponsor and a renewal owner would have done, alongside her clinical advocacy — without the authority or the explicit accountability of either role, which is why the table shows those rows as never assigned. She could carry the champion's seat; she was never positioned to carry the other two. When she moved, the informal work stopped, and the gap between effort and assigned authority became visible in the same week.

Which function will resist hardest, and is it about the product or the burden? The triage nurses — and it was about the burden, and possibly about the product too. The vendor's metric was true in its own terms: drafting time fell. The nurses' experience was also true: each draft had to be read against the original message, corrected in about a third of cases (an illustrative input of this scenario), and routed anyway. The scenario does not supply the end-to-end time and quality measurements that would decide the net effect, so the honest statement at month four is that work moved and nobody had measured whether any was removed. Nobody had mapped where the work would land, so nobody noticed when it landed on the people with the least slack. The nurses' experience is evidence to investigate, not resistance to explain away.

Who will be in the renewal meeting, and what will they remember? Nobody was scheduled to be in it. If it happens, the signer will remember that it was free, the implementer will remember the exception queue, the nurses will remember month four, and the chief medical officer will remember being asked to own something in month seven that she never agreed to in January.

Restarting, if the institution chooses to

The stack says what has to be true before the pilot resumes. All seven seats first (Instrument 1, updated), then the four-name check (Instrument 5), then the charter (Instrument 8), then the safety words walked back to their mechanisms (Instrument 6). And one test the first attempt skipped: whether the product is good enough at this institution's messages to be worth the work — measured before a patient is on the other end of it, not inferred from the fact that the governance had failed.

Instrument 1, updated — all seven seats, by name, before restart (illustrative). One person may hold more than one seat where the combination is explicit and workable; the sheet says so where it happens.

Seat	Who, by name	What they accepted	Authority and capacity
Champion	Dr. Samuel Adeyemi, regional medical director for primary care (Dr. Okafor's successor)	The champion's role, in writing: the clinical case, the nurses' confidence, the weekly review of the audit	Four protected hours a week for the pilot's 120 days; not the renewal owner, by design
Sponsor	Dr. Helen Marsh, chief medical officer	The role, in writing, with the burden map, the Instrument 6 architecture and the renewal plan in hand; the title did not fill the seat — the acceptance did	Owns the decision and its trade-offs; can stop the pilot
Signer	Marcus Bell, vice president of supply chain	To sign only against a year-two price and a data-use clause the privacy officer, Grace Lindqvist, has read and approved	Authorizes the contract; does not decide clinical scope
Budget owner	Ana Ferreira, vice president of ambulatory operations	The pilot's internal resource budget (the nurse hours, the informatics time, the audit) and the funding side of any continuation	Holds the ambulatory operating budget the costs land in
Implementer	Priya Natarajan, ambulatory informatics manager	The charter, the integration plan, the exception path, the training plan — written before activation this time	Authority to pause the tool on burden, quality or safety criteria without asking first
User(s)	Carmen Delgado, RN, triage nurse lead at the largest clinic, and a medical-assistant lead from the second clinic	To be in the room for every decision that touches the queue, and to run the weekly audit sample	Protected time for the audit; a direct line to the implementer
Renewal owner	Ana Ferreira (also the budget owner — one person, two seats, written down)	The day-120 decision's evidence, date and agenda; the renewal case is deliberately not carried by the champion, so it does not depend on the person who most wants the tool	Owns the continuation decision's funding and timing with the sponsor

Instrument 5 — the four-name check, after the seven. Sponsor: Dr. Marsh. Signer: Mr. Bell. Implementer: Ms. Natarajan. User: Ms. Delgado. Then the test: how many of the four believe they are the buyer? In January, one; in the restart, all four know which one is — the sponsor owns the decision, the signer authorizes the contract, and neither of them is the user. The quick check does not replace the full map; it confirms that the four seats the restart most depends on are real people who have agreed, in writing, to what the seat requires.

Instrument 8 — the charter, six lines, before the restart. Every value below is illustrative, chosen for the example; a real institution's clinical leadership sets its own. Any blank cell — an owner unnamed, a threshold unagreed, a stop condition undefined — keeps the restart on HOLD.

Charter line	Entered (illustrative)
Named owner — institution side	Dr. Helen Marsh, chief medical officer (sponsor; the decision) and Priya Natarajan, ambulatory informatics manager (operational owner; can pause on burden, quality or safety criteria)
Named owner — vendor side	the vendor's implementation lead, by name, with a named successor — both recorded in the charter

Charter line	Entered (illustrative)
Scope and end date	Three clinics (the largest, a mid-sized one, a small one); two administrative message types only — appointment requests and the administrative intake of refill requests; 120 days from activation; end date on the calendar; the day-120 meeting booked now. The tool proposes an acknowledgment and a destination queue. It does not approve a refill, recommend treatment or assign clinical urgency, and a nurse releases every reply and every routing
Success criteria — including burden criteria — agreed in writing before go-live	(1) Nurse time per in-scope message, measured end to end — read the original, review the draft, correct it, route it — baseline median 4.0 minutes over the four weeks before activation in the three clinics; target: median at or under 3.0 minutes over days 61–120, measured the same way, exceptions included. (2) Correctness, measured independently of acceptance: a weekly audit of 50 released messages per clinic (150 a week; about 2,500 over the pilot), read against the original message by a nurse auditor who did not release them; material errors — wrong queue, wrong appointment detail, a clinical element in the message missed — in no more than 2 percent of audited messages; the acceptance-without-edit rate is reported but is not a success criterion, because a nurse can accept a wrong draft. (3) Intake boundary: any symptom-containing or ambiguous message that the tool labeled administrative and that reached release without being redirected is reviewed within one business day by the champion and the implementer; one such finding triggers a scope review; two in any 30 days pause that message type. (4) Nurse-reported burden: a five-question survey of the three clinics' triage staff monthly, not rising against the pre-activation baseline; the implementer can pause on this criterion alone
Measurement plan finance has blessed in advance	the inbox time report finance accepted — end-to-end nurse time, not the vendor's drafting-time figure; the weekly audit log; the routing-error log; the monthly survey; the pre-activation shadow report; assembled by the implementer's analyst and the nurse auditor, reported to the sponsor monthly and to the governance committee
Decision path: what converts the pilot, what kills it, and who makes that call	Day 120, on the calendar now: Dr. Marsh decides, with Ms. Ferreira (budget and renewal owner) and Ms. Natarajan in the room. Criteria met → consideration of a priced continuation proposal for the three clinics; expansion to more clinics or message types needs its own charter and is not automatic. Any burden criterion failed → stop. A safety event, a privacy incident or an exceeded stop threshold pauses the tool the day it is found — the implementer or the privacy officer can pause, and the sponsor is told the same day

Instrument 6 — the safety words, walked back to their mechanisms. *Accountable:* a named nurse releases every reply and every routing; the review surface shows the original message, the draft and the proposed queue side by side; nothing leaves without a human release. The tool's own label never decides eligibility: every incoming message still enters the clinic's ordinary intake and escalation pathway first, the tool works only on the messages that pathway has already placed in the administrative queues, and a nurse can send any message back to the clinical pathway at any point. *Transparent:* every draft, every edit and every release is logged so that a misroute can be reconstructed after the fact. *Monitored:* the weekly audit, the misroute rate and the intake-boundary findings are measured against the charter's thresholds, with a named person — the implementer, with the nurse lead — who receives the alert; the privacy officer receives data-flow alerts. *Governed:* the governance committee can narrow the scope, suspend a message type, pause the tool and audit the log; it can explain all of that to a board. *Compliant:* the business associate agreement — the vendor handles protected health information, and the fact that the pilot was free never changed that — minimum-necessary data flows, the breach obligations, and a data-use clause that says what the vendor may and may not do with the messages (no training on them without permission; retention and deletion at the end), all dated before the restart. *Validated:* before any patient-facing activation, a local evaluation on a defined set of this system's past messages, under the governance committee's approval and the agreement, with symptom-containing and ambiguous messages deliberately mixed into the administrative set to see whether the intake pathway and the tool catch them; then a two-week shadow phase in the three clinics in which the tool drafts but nothing it produces reaches a patient or changes a routing, scored by the nurse auditor against the charter's thresholds; activation only if the shadow results meet them and the intake pathway caught every planted out-of-scope message; then the weekly audit through the pilot; the scope widens only with new numbers in hand. A finite test with zero severe errors does not prove zero future risk; it earns a bounded start. A vendor fluent only at the document level has compliance theater; structure, process, outcome, or it is language.

What this example teaches

A free pilot entered the building without a sponsor, a budget owner or a renewal owner, so when its champion left there was no one left to hold it, and the burden it moved landed on the one group that had never been asked. The worksheet would have shown all of that in January, in fifteen minutes, on one page. It does not say whether the tool was good; it says the institution had never built the structure that could find out. The restart builds it — seven seats with names, a charter with numbers, a safety architecture, and a test of the product before a patient is on the other end of it. Zero on the invoice never meant zero in the building, and it never meant the evidence could be skipped.

These instruments are aids to structured thinking based on the author's operating experience. They are not legal, clinical, compliance, investment, accounting, tax, or consulting advice; they do not establish a standard of care or guarantee diligence or outcomes. Use them with qualified professionals and your own judgment. This worked example is a hypothetical composite; all names, organizations and figures are illustrative.